SACTCHECK · TREATMENT PASSPORT

Nivolumab and ipilimumab for advanced melanoma

A record of what you experience

One cycle per page. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

The combination phase gives nivolumab followed by ipilimumab by intravenous (IV) infusion every three weeks for up to four doses. If appropriate, nivolumab alone then continues on the team-selected two-weekly or four-weekly schedule, by the prescribed IV or subcutaneous route. After the last combination dose, the first two-weekly maintenance dose starts three weeks later; the first four-weekly maintenance dose starts six weeks later in this melanoma schedule. Treatment continues until progression or unacceptable toxicity, subject to review.

These pages are reusable recording sheets; the prescribed course length is stated above. Ask for additional sheets as needed.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Your own hospital / contact instructions:

https://sactcheck.com/patient/00431/

Full patient information, sources and visual timeline. No identifiers are encoded.

Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending

My diagnosis, plan and medicines

Complete these pages with your team. Leave unknown or inapplicable items blank. This record supports the consent discussion and your written prescription.

Diagnosis and overall plan

Original melanoma site / pathology date

Stage / Breslow thickness / ulceration

Lymph-node or other disease sites

BRAF result / date / other relevant tumour tests

Advanced or adjuvant intent / earlier surgery or radiotherapy (RT)

Medicines and safety

Anticancer medicines / actual dose / route / start date

Treatment received, missed or held / restart instruction

All other medicines / allergies / important previous reactions

Pregnancy-prevention or immune-alert plan / continuing precautions

Oncology / out-of-hours / pharmacy contacts

SACTCheck patient information. Your team confirms your personal plan. The treatment reference collects the clinical and medicine sources.

My checks, symptoms and support

Record what each test is checking and when the result will be discussed. Your team explains which examinations or scans are needed for your diagnosis and treatment.

Test and purpose	Date / booking contact	Result discussion	Agreed next step

Record bloods, skin/lesion examinations, scans and specialist checks separately. Tumour-marker and scan intervals are individually selected; do not infer them from the cycle calendar.

Symptoms, skin and support

New symptoms / onset / change / action taken

Lesion or wound / pain / eating / weight / daily function

Skin self-check plan / optional dated photograph record kept privately

Sun protection / practical, emotional or family support needs

Questions, response review and the outcomes that matter to me

My urgent contact plan / what my carer should do

SACTCheck patient information. Your team confirms your personal plan. The treatment reference collects the clinical and medicine sources.

Nivolumab and ipilimumab for advanced melanoma

Cycle number: _____ Start date: _____ Next review: _____

Record the actual date, treatment received, any hold, important symptom and the next check. A review visit is not automatically a medicine dose or a completed cycle.

Combination induction: doses 1–4 · copy this sheet for another cycle of the same phase.

Nivolumab · PD-1 checkpoint immunotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Bowels	□ Usual pattern	□ New change: report	□ Persistent diarrhoea / constipation: call	□ Blood, severe pain or dehydration: urgent review	□ Collapse or severe sudden illness: 112/999
Breathing	□ Usual breathing	□ New cough / change: call	□ Worsening breathlessness: urgent advice	□ Breathless at rest: urgent assessment	□ Severe breathing difficulty: 112/999
Skin	□ No change	□ New small change: report	□ Persistent rash / itch: contact team	□ Painful, spreading or blistering rash: urgent review	□ Severe skin reaction with acute illness: emergency help
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999

Ipilimumab · CTLA-4 checkpoint immunotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Diarrhoea	□ Usual pattern	□ Up to 3 extra stools above normal	□ 4–6 extra stools/day or at night: call	□ 7+ extra stools/day or dehydration: urgent review	□ Collapse / severe dehydration: 112/999
Abdominal pain	□ None	□ New pain: contact team	□ Persistent / worsening pain: urgent advice	□ Severe pain, blood or repeated vomiting: urgent assessment	□ Collapse or sudden severe illness: 112/999
Headache	□ None	□ Mild and short-lived: record	□ New / persistent headache: call	□ Severe headache / visual change: urgent help	□ Sudden worst headache / weakness: 112/999
Weakness	□ Usual strength	□ New tiredness: record	□ New weakness limiting tasks: call	□ Marked weakness / faintness: urgent review	□ Sudden weakness, confusion or collapse: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

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Cycle number: _____ Start date: _____ Next review: _____

Record the actual date, treatment received, any hold, important symptom and the next check. A review visit is not automatically a medicine dose or a completed cycle.

Nivolumab maintenance: interval and route as prescribed · copy this sheet for another cycle of the same phase.

Nivolumab · PD-1 checkpoint immunotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Bowels	□ Usual pattern	□ New change: report	□ Persistent diarrhoea / constipation: call	□ Blood, severe pain or dehydration: urgent review	□ Collapse or severe sudden illness: 112/999
Breathing	□ Usual breathing	□ New cough / change: call	□ Worsening breathlessness: urgent advice	□ Breathless at rest: urgent assessment	□ Severe breathing difficulty: 112/999
Skin	□ No change	□ New small change: report	□ Persistent rash / itch: contact team	□ Painful, spreading or blistering rash: urgent review	□ Severe skin reaction with acute illness: emergency help
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.